



BORNO STATE GOVERNMENT

**REPORT OF BASELINE MAPPING EXERCISE
FOR
PRIMARY HEALTHCARE WORKERS
IN BORNO STATE**

PREPARED BY:

**BORNO STATE MINISTRY OF HEALTH AND HUMAN SERVICES
AND
BORNO STATE PRIMARY HEALTHCARE DEVELOPMENT BOARD**

FEBRUARY 2025

Table of Contents

List of Acronyms	4
1.0 INTRODUCTION	8
1.1 Background	8
1.2 Objectives of the Baseline Mapping Exercise	9
1.3 Scope of the Baseline Mapping Exercise	9
1.4 Methodology	9
1.4.1 Approach	9
1.4.2 Data Collection Method	10
1.4.3 Methodological Assumption and Limitations	10
1.4.4 Validation	10
1.4.5 Methodological Assumptions	11
2.0 SITUATIONAL ANALYSIS	12
2.1 Overview of Borno State's Primary Healthcare System and Human Resources	12
2.1.1 Number of Government-owned PHC Facilities in Borno State	12
2.1.2 Health Facility Density in Borno State	13
2.1.3 Location of Health Facilities in Borno State and Rural-Urban Continuum	14
2.1.4 Distribution of Health Facility by Type	15
2.1.5 Current Healthcare Workforce in Borno State and their Location	16
2.1.5.1. Total Healthcare Workforce in Borno State by Location	16
2.1.5.2 Distribution of Health Workers by Gender	17
2.1.5.3 Average Health Worker per Facility	18
2.1.5.4 Distribution of Health Workers by Job Area	19
2.1.5.5 Distribution of Health Workers by type of Duty Station (Rural-Urban Continuum)	23
2.1.5.6 Analysis of Workers Leaving the Service Between 2025 and 2029	23
2.2 Staff Gap Analysis and Findings	24
2.2.1 Total PHC Staff Gap by Professional Area in Borno State	24
2.3 Institutional Structure for Primary Health Human Resource Management in Borno State	26
2.3.1 Minimum Standard for PHC Workers	26
2.3.2 Gender and Social Inclusion Requirements in Borno State	26
2.4 Challenges of Health Workers Recruitment, Deployment and Retention in Borno State	27
3.0 RECRUITMENT/DEPLOYMENT PLAN AND COSTING	29
3.1 Recruitment Plan	29
3.1.1 PHC Workers Recruitment Plan by Job Area (5-year Projection)	29
3.1.2 Workers Recruitment Plan by Classification of Duty Station	30

3.1.3 PHC Workers Recruitment by Duty Station (PHC Facility)	31
3.1.4 Borno State PHC Staffing Recruitment Strategy	31
3.2 Deployment plan	32
3.2.1 PHC Workers Deployment Plan by Classification of Duty Station	32
3.2.2 Framework for Equitable Workers Deployment	33
3.3 Multi-Year Costed PHC Workers Recruitment and Deployment Plan	33
3.4 Recruitment Calendar	34
3.5 Integration with the State Health Annual Operational Plan	35
4.0 RECOMMENDATIONS	36
5.0 CONCLUSION	37

List of Acronyms

ANC	Antenatal Clinic
AOP	Annual Operation Plan
BHCPF	Basic Health Care Provision Fund
CHEW	Community Health Extension Worker
CHO	Community Health Officer
CHW	Community Health Worker
DFF	Direct Facility Financing
DPHC	Directors of Primary Health Care
FMOH	Federal Ministry of Health
GRA	Government Reserved Area
HC	Health Clinic
HIS	Health Information System
HMB	Hospital Management Board
HMIS	Health Management Information System
HND	Higher National Diploma
HP	Health Post
HOPE-GOV	Human Capital Opportunities for Prosperity and Equity - Governance
HOPE-PHC	Human Capital Opportunities for Prosperity and Equity - Primary Healthcare
HRH	Human Resource for Health
HRIS	Human Resource Information System
HSP	Health Service Post
JCHEW	Junior Community Health Extension Worker
LGA	Local Government Area
MDG	Millennium Development Goals
MDAs	Ministries, Departments, and Agencies
MMC	Maiduguri Metropolitan Council
MSP	Minimum Service Package
NA	Not Applicable
NPHCDA	National Primary Health Care Development Agency
PHC	Primary Healthcare
PHCC	Primary Health Care Clinic
PHCUOR	Primary Health Care Under One Roof
UHC	Universal Primary Coverage
RM	Registered Midwife
RN	Registered Nurse
SHDP	Strategic Health Development Plan
SMOH	State Ministry of Health
SPHCDB	State Primary Healthcare Development Board
WHO	World Health Organization

Foreword

The health and wellbeing of our citizens sit at the very centre of Borno State's development agenda. Primary Health Care is the cornerstone of our health system — the first point of contact for millions of residents across our urban and rural communities — and the Borno State Government has remained steadfast in its commitment to strengthening it. Central to that commitment is investing in the workforce that delivers it.

In recognition of this, the Ministry of Health and Human Services, working in close collaboration with the State Primary Health Care Development Board, commissioned this baseline mapping exercise to establish a clear, evidence-based picture of our PHC workforce. The result is a report that is both timely and actionable — one that reflects the State's commitment to transparency, evidence-led planning, and accountability in health sector governance.

The report documents the current staffing position across our 329 PHC facilities, benchmarks it against the National Primary Health Care Development Agency (NPHCDA) minimum standards, and projects future workforce requirements considering anticipated retirements and population growth. Where gaps exist, this report names them clearly, because addressing them begins with understanding them. These findings will directly inform the development of a costed, multi-year recruitment and deployment strategy that is equitable, sustainable, and built around the needs of our communities.

I take this opportunity to acknowledge the extraordinary dedication of our frontline health workers, who continue to serve communities across Borno State with professionalism and commitment — often under demanding conditions. It is precisely because of their service that the State Government is determined to give them the support, resources, and workforce environment they deserve.

We are equally grateful to our partners — the National Primary Health Care Development Agency, the Ministry for Local Government and Chieftaincy Affairs, and our development partners — whose collaboration and technical support have been integral to this exercise and to our broader health sector reform agenda.

As Commissioner for Health, I affirm that the findings of this report will be translated into concrete action. Borno State will ensure that workforce recruitment costs are captured in each subsequent annual budget, and that adequate provisions are made for continuous training, professional development, and strategic redeployment. Our goal is unambiguous: a PHC system that meets national and global standards, and that ensures every ward in Borno State has access to quality, well-staffed health services.



Professor Baba Malam Gana

**Honorable Commissioner,
Borno State Ministry of Health and Human Service**

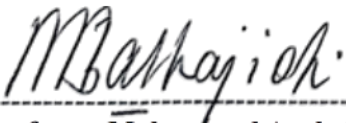
Acknowledgment

On behalf of the Borno State Primary Health Care Development Board, I wish to extend my deepest gratitude to all who contributed to the successful completion of this baseline mapping exercise of primary health care workers in our State. This report is the product of collective effort, and it reflects the commitment of many institutions and individuals to strengthening our health system.

We are especially grateful to the Borno State Ministry of Health and Human Services for their leadership and technical guidance throughout the process. We also acknowledge the invaluable support of the Borno State Ministry for Local Government and Chieftaincy Affairs, whose collaboration ensured that local authorities and traditional institutions were fully engaged in providing accurate data and facilitating access to communities. Their partnership has been critical in aligning this exercise with the realities of our grassroots governance structures.

Special appreciation goes to the Directors of Primary Health Care across the 27 Local Government Areas, the Officers-in-Charge of PHC facilities, and our dedicated survey teams, whose tireless work and cooperation made data collection possible even under challenging circumstances. We also recognize the National Primary Health Care Development Agency (NPHCDA) and our development partners for their technical assistance and continued support in advancing human resources for health reforms.

Finally, I commend the resilience and dedication of our frontline health workers, who continue to serve the people of Borno State despite resource constraints and security challenges. This report is dedicated to them, and it is our hope that the recommendations herein will translate into tangible improvements in their working conditions and in the quality of care delivered to our communities.



Professor Mohammed Arab Alhaji

Executive Secretary

Borno State Primary Healthcare Development Board (SPHCDB)

Executive Summary

The Borno State Ministry of Health and Human Services, working in close collaboration with the State Primary Health Care Development Board, conducted a comprehensive baseline mapping of Primary Health Care (PHC) workers across the State in February 2025. This exercise reflects the State Government's commitment to evidence-based health sector planning. Using a rigorous mixed methodology — combining desk review, structured surveys, and stakeholder consultations — the mapping established a credible workforce profile across all 27 Local Government Areas (LGAs), capturing the distribution, composition, and deployment of the PHC workforce and providing the foundation for a costed, multi-year recruitment strategy and deployment plan benchmarked against the National Primary Health Care Development Agency (NPHCDA) minimum standards.

Borno State operates 329 registered Primary Health Care facilities distributed across its 27 LGAs and 312 wards. In acknowledgement of the impact of the protracted security crisis on service delivery infrastructure, the State has taken a realistic and transparent approach to its planning baseline: of the 329 registered facilities, 274 wards currently have operational facilities, while 38 wards remain without an active PHC presence as a direct consequence of the conflict. The baseline mapping was conducted across all 329 facilities to ensure a complete picture of the State's PHC asset base, and the gap analysis is anchored on the 274 operational wards — reflecting the State's commitment to working from facts, not assumptions.

The strategic focus of this report goes beyond counting facilities. It assesses their functional adequacy against the NPHCDA standard of at least one functional PHC per ward and sets out a clear pathway for upgrading all facilities in the 274 operational wards to that standard. In wards where more than one facility exists, the additional facilities will continue to operate in a supplementary capacity — preserving service continuity while the upgrading programme brings each ward's primary facility to the required standard.

The mapping found that Borno State's PHC network is staffed by 3,916 workers across its 329 facilities. Of these, 35% are core health professionals — including doctors, nurses, midwives, Community Health Extension Workers (CHEWs), Junior Community Health Extension Workers (JCHEWs), laboratory and pharmacy technicians, and medical records officers — with the remaining 65% comprising support staff. The State recognizes that workforce distribution across rural and urban LGAs requires deliberate rebalancing, and that gender disparities — particularly the dominance of male staff in rural areas — must be addressed as part of an equitable deployment strategy. Benchmarked against NPHCDA minimum staffing standards, and accounting for projected retirements of 758 workers between 2026 and 2030 as well as population growth projections, the State has identified a total staffing gap of 2,628 workers to be filled over the next five years. This is a challenge the State Government is prepared to meet head-on.

To address this gap in a structured and sustainable manner, this report sets out a phased recruitment and deployment plan that prioritises cadres with the most acute shortages and directs 77% of new hires to rural facilities — reflecting the State's commitment to health equity. The plan is complemented by strategies to redistribute surplus cadres, regularise existing volunteers, and strengthen staff retention through targeted incentives including housing support, allowances, and clear pathways for career progression.

The financing requirement for this plan is estimated at ₦350 million in 2026, rising to ₦572 million in 2027 — a level of investment the State Government is committed to reflecting in its annual budgets. Successful implementation will be underpinned by governance reforms under the Primary Health Care Under One Roof (PHCUOR) framework and a robust monitoring and accountability mechanism to track progress against agreed targets. Implemented with sustained political will and fiscal commitment, this strategy will position Borno State to progressively meet national staffing standards, deliver more equitable health services across all LGAs, and build a resilient, well-staffed PHC system capable of serving the State's growing population — now and into the future.

1.0 INTRODUCTION

The strength of any health system is inseparable from the caliber and organization of its workforce. Human resources for health, according to the World Health Organization (WHO), include all individuals who promote and preserve health, diagnose illnesses, and deliver medical care. Human resources for health encompass the diverse professionals who safeguard well-being, deliver care, and respond to community health needs. When this workforce is effectively managed and equitably deployed, the benefits are far-reaching, with improved access to primary care, stronger immunization coverage, and measurable reductions in maternal and child mortality. Beyond their clinical impact, health workers represent a substantial investment within state budgets, making their management a cornerstone of sustainable health system performance.

In Borno State, the imperative for a coherent and well-supported workforce strategy is particularly evident. The State's primary health care facilities must contend with both shortages and uneven distribution of personnel, challenges that compromise service delivery in rural and hard-to-reach areas. Addressing these gaps requires deliberate planning, fair deployment, and continuous professional development to ensure that every community has access to quality care. Reliable workforce data and structured management frameworks are essential to guide recruitment, training, and retention. By prioritizing these reforms, Borno State can strengthen its primary health care system, reduce dependence on external health services, and build a resilient foundation for improved health outcomes and broader socio-economic progress.

1.1 Background

Human resources for health remain one of the most critical challenges facing Nigeria's health sector, particularly at the primary health care level. Persistent shortages of skilled personnel, uneven distribution across urban and rural facilities, and fragmented management structures have consistently undermined service delivery across the country. In Borno State, these challenges are compounded by the rise of insecurity which limits the availability of frontline workers and makes it increasingly difficult to maintain minimum staffing standards across the 329 operational PHC facilities.

To address these systemic gaps, the Nigeria Human Capital Opportunities for Prosperity and Equity – Governance (HOPE-Gov) Program for Results (PforR) requires States to undertake a baseline mapping of primary health care workers. This exercise provides credible data on the

number and duty stations of PHC workers and forms the foundation for a costed, multi-year recruitment and deployment plan. By establishing a reliable workforce baseline, states can strengthen human resource planning, promote equitable distribution of personnel, and systematically close staffing gaps. The HOPE-Gov framework ensures that recruitment and deployment decisions are evidence-based, transparent, and aligned with broader goals of improving access to quality primary health care services.

1.2 Objectives of the Baseline Mapping Exercise

The objectives of the baseline mapping exercise were to:

- i. Ascertain the total number of staff in primary healthcare facilities in the state.
- ii. Identify human resources gaps in primary health care facilities in the state.
- iii. Develop and propose a recruitment and deployment strategy and plan for primary healthcare human resources in the state.

1.3 Scope of the Baseline Mapping Exercise

The geographic scope encompasses the full administrative territory of the State, which is 312 wards across 27 LGAs. All 329 operational PHC facilities across the 27 LGAs were included in the data collection and documentation phase of the mapping exercise. However, due to the rise of insecurity in certain regions of the State, the operational gap analysis and the upgrading programme focuses on 274 wards.

1.4 Methodology

1.4.1 Approach

The baseline survey adopted a mixed survey method, leveraging desk research and existing database reviews for secondary data and the use of a survey form to triangulate and update the data collected from secondary sources. The exercise uses interviews and focus group discussions with relevant stakeholders to develop and validate a realist recruitment and deployment plan based on the PHC staff gap identified.

1.4.2 Data Collection Method

Preliminary data for the health workers' mapping and baseline was obtained from the State's Human Resources for Health Office at the Borno State Ministry of Health and complemented with primary data collection using a structured data form presented in Annex 2 of this report. The data collected from the State's Human Resources for Health Office was as of March 2024, being the most recent data collection point. This data was triangulated using the survey data for accuracy and credibility. In addition, data was also sourced from the Borno State Health Profile (2025 edition) and other related secondary sources to strengthen the evidence base and ensure comprehensive coverage of the State's healthcare workforce situation.

1.4.3 Methodological Assumption and Limitations

Data collection was done at the LGA health authorities, which led to rigorous data cleaning and sorting. This method was likely to leave out some staff whose data was not available to the LGA health authorities at the time of data collection and triangulation and facilities in wards that are not accessible due to insurgency and security concerns. The best method would have been to visit all the health facilities to collect the data, but this was not done due to time, security and financial constraints.

1.4.4 Validation

To guarantee the accuracy and dependability of the baseline mapping report, a range of validation activities were carried out. Spot checks were undertaken in selected health care facilities across the local government area to ensure that reported data matched on-the-ground realities. The information gathered was then cross-verified with both state and local government payroll records, the State Human Resource for Health database, and the State Annual Operational Plan. These measures created a solid framework for confirming the consistency and reliability of the dataset. The process concluded with a two-day stakeholder engagement that brought together facility management committees, ward development committees, health-focused civil society organizations, and facility in-charges. This participatory forum enabled stakeholders to examine the findings, contribute contextual insights, and supply additional evidence, leading to actionable recommendations for closing identified gaps. Their collective contributions reinforced the credibility of the baseline report and positioned it as a guiding tool for strengthening primary health care delivery in Borno State.

1.4.5 Methodological Assumptions

The baseline mapping exercise is built on several key assumptions: that workforce projections based on the March 2024 Human Resource for Health database, NPHCDA minimum staffing standards and population growth estimates remain valid throughout the planning period; retirements will occur at the statutory age of 60, with 758 staff expected to exit the workforce between 2026 and 2030; and the state's annual population growth rate of 3.98% will push Borno's population to approximately 9.2 million by 2030, necessitating additional health workers to meet service delivery needs.

2.0 SITUATIONAL ANALYSIS

2.1 Overview of Borno State's Primary Healthcare System and Human Resources

Borno State is administratively organized into 27 Local Government Areas (LGAs), which are further subdivided into 312 wards. As of the time of this mapping exercise, the State has a total of 329 operational PHC facilities distributed across these LGAs. These facilities span the three tiers of the primary health care system: Health Posts, Primary Health Clinics (PHCs) and Primary Health Care Centres (PHCCs). The Primary Health Care (PHC) system in Borno State, as in other parts of Nigeria, represents the third level of health care delivery. Its primary role is to provide preventive services such as community mobilization, health education, personal hygiene, environmental sanitation, immunization, and early disease outbreak notification.

The system is coordinated by the Executive Secretary of the State Primary Health Care Development Board (SPHCDB), who oversees daily operations and ensures alignment with national standards. At the local government level, Directors of Primary Health Care (DPHCs) manage PHC activities across the 27 LGAs, reporting directly to the SPHCDA. At the facility level, officers-in-charge, experienced community health extension workers (CHEWs), are responsible for service delivery and report to their respective DPHCs.

Urban LGAs tend to have relatively higher numbers of staff, while rural and hard-to-reach LGAs struggle with minimal personnel. Borno State has made measurable progress in building its PHC workforce across several cadres. Building on this foundation, the State is committed to addressing workforce distribution imbalances through targeted recruitment, strategic redeployment, and strengthened retention measures — ensuring that every tier of the PHC system is equipped to deliver quality care to all communities across the State.

2.1.1 Number of Government-owned PHC Facilities in Borno State

As illustrated in the chart below, Borno State currently has 329 operational health facilities across 274 wards. The remaining 38 wards have no active PHC facility, primarily because of the security situation in the State — a consequence of the insurgency that has displaced communities, damaged infrastructure, and made certain areas inaccessible for health service delivery. These 38 wards are distributed across several LGAs in the most conflict-affected zones of the State. While they are

excluded from the operational gap analysis, restoring PHC access to these wards remains a medium- to long-term priority for the State.

The distribution across the 274 wards is such that Shani LGA has the highest number of facilities (34), followed by Maiduguri Metropolitan Council (MMC) with 31 facilities. In contrast, Kala Balge, Marte, and Biu LGAs have the lowest coverage. A proportion of these facilities benefit from Direct Facility Financing (DFF) under the Basic Health Care Provision Fund (BHC PF), which supports service delivery at the facility level. Detailed information on these supported facilities is captured in the State's PHC facility database, serving as a critical tool for planning, resource allocation, and ongoing health system strengthening across all LGAs.

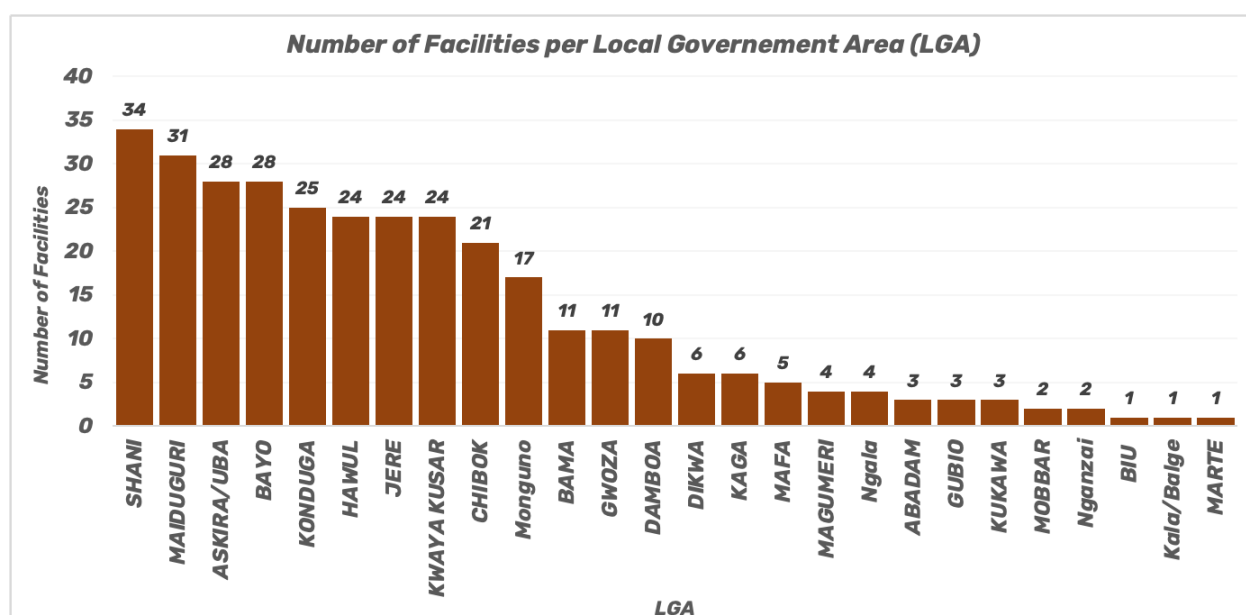


Figure 1: Chart showing Numbers of Health Facilities per LGA
Source: Borno State Health Workforce Data 2024

2.1.2 Health Facility Density in Borno State

Health facility density measures the availability of health infrastructure by estimating the number of public health facilities per 10,000 population. It is a key proxy for assessing access to essential outpatient healthcare services within a population. In 2025, Borno State is projected to have a population of approximately 6.9 million. With 329 government-owned operational health facilities spread across its 27 Local Government Areas (LGAs), the state's health facility density is estimated at 0.8 per 10,000 population. This implies that, on average, one health facility serves about 12,500 residents.

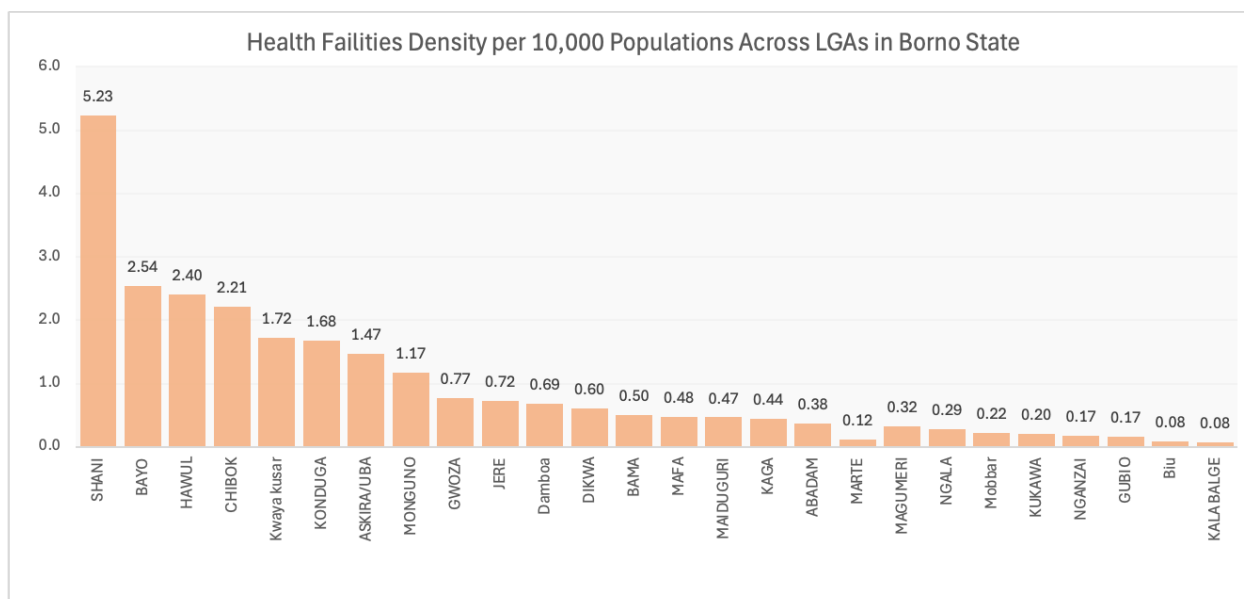


Figure 2: Health Facility Density per LGA

Source: Borno State Health Workforce Data 2024

2.1.3 Location of Health Facilities in Borno State and Rural-Urban Continuum

Out of the total 329 government-owned operational health facilities, approximately 254 facilities (77%) are situated in rural communities, while 75 facilities (23%) are in urban centers. As illustrated in Figure 3, Maiduguri Metropolitan Council (MMC) and Jere have the highest number of PHC facilities among the urban LGAs, with 31 and 24 facilities, respectively. In addition, Shani, Bayo, and Askira/Uba also show a relatively higher concentration of facilities within their urban areas.

This pattern of distribution is significant for planning under the Basic Health Care Provision Fund (BHCPF), particularly in the implementation of Direct Facility Financing (DFF), which focuses on improving service delivery in underserved communities. Furthermore, the rural-urban distribution provides critical insights for effective workforce allocation, logistics planning, and the design of targeted community outreach strategies across the state.

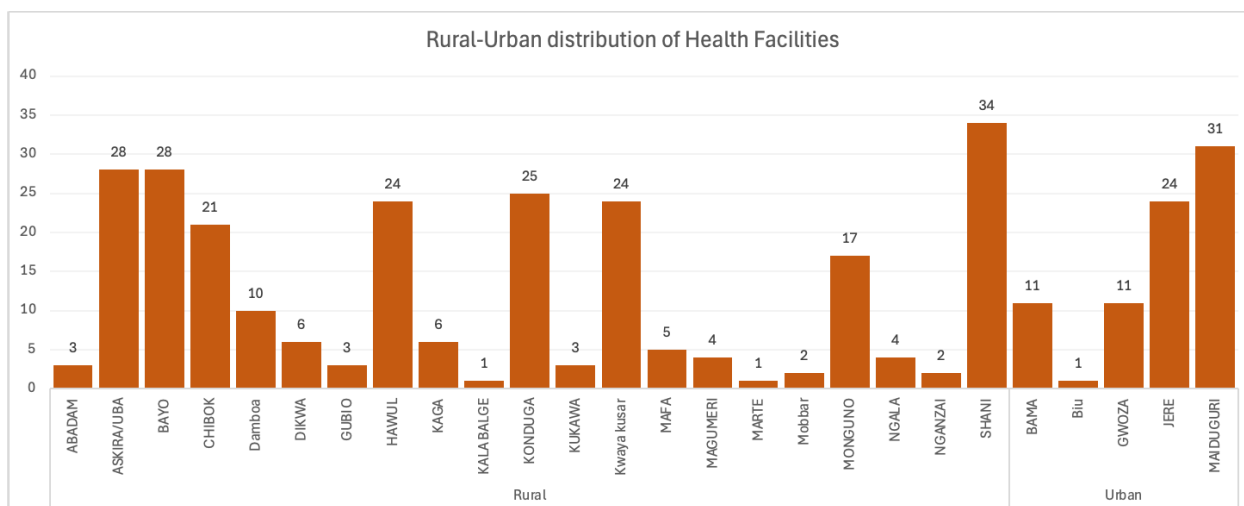


Figure 3: Rural-Urban Distribution of Health Facilities
Source: Borno State Health Workforce Data 2024

2.1.4 Distribution of Health Facility by Type

Figure 4 shows that Primary Health Clinics (PHCs) constitute the largest proportion of facilities at 45.0% (148), followed closely by Primary Health Care Centers (PHCC) at 42.2% (139), while Health Posts account for a much smaller share at 12.8% (42).

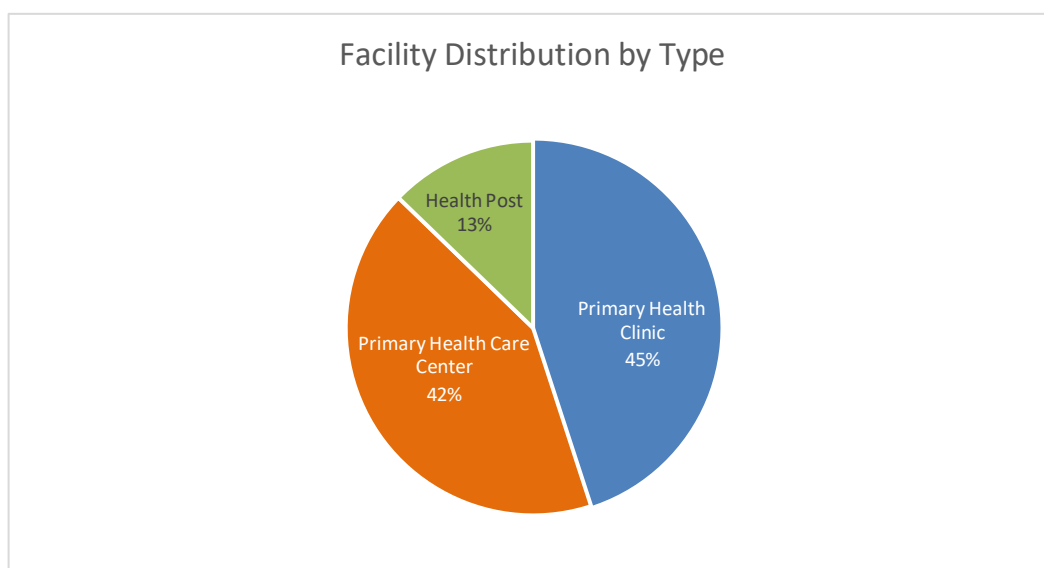


Figure 4: Facility Distribution by Type
Source: Borno State Health Workforce Data 2024

2.1.5 Current Healthcare Workforce in Borno State and their Location

2.1.5.1. Total Healthcare Workforce in Borno State by Location

There are 329 operational healthcare facilities across Borno State which are staffed by a total of 3,916 staff. In terms of absolute numbers, Maiduguri Metropolitan Council (MMC) has the highest number of staff (531), which is rather consistent with its relatively high number of healthcare facilities (31) and its urban classification and attendance population. Kala Balge Local Government, on the other hand, has the least number of staff (4), as shown in Figure 4.

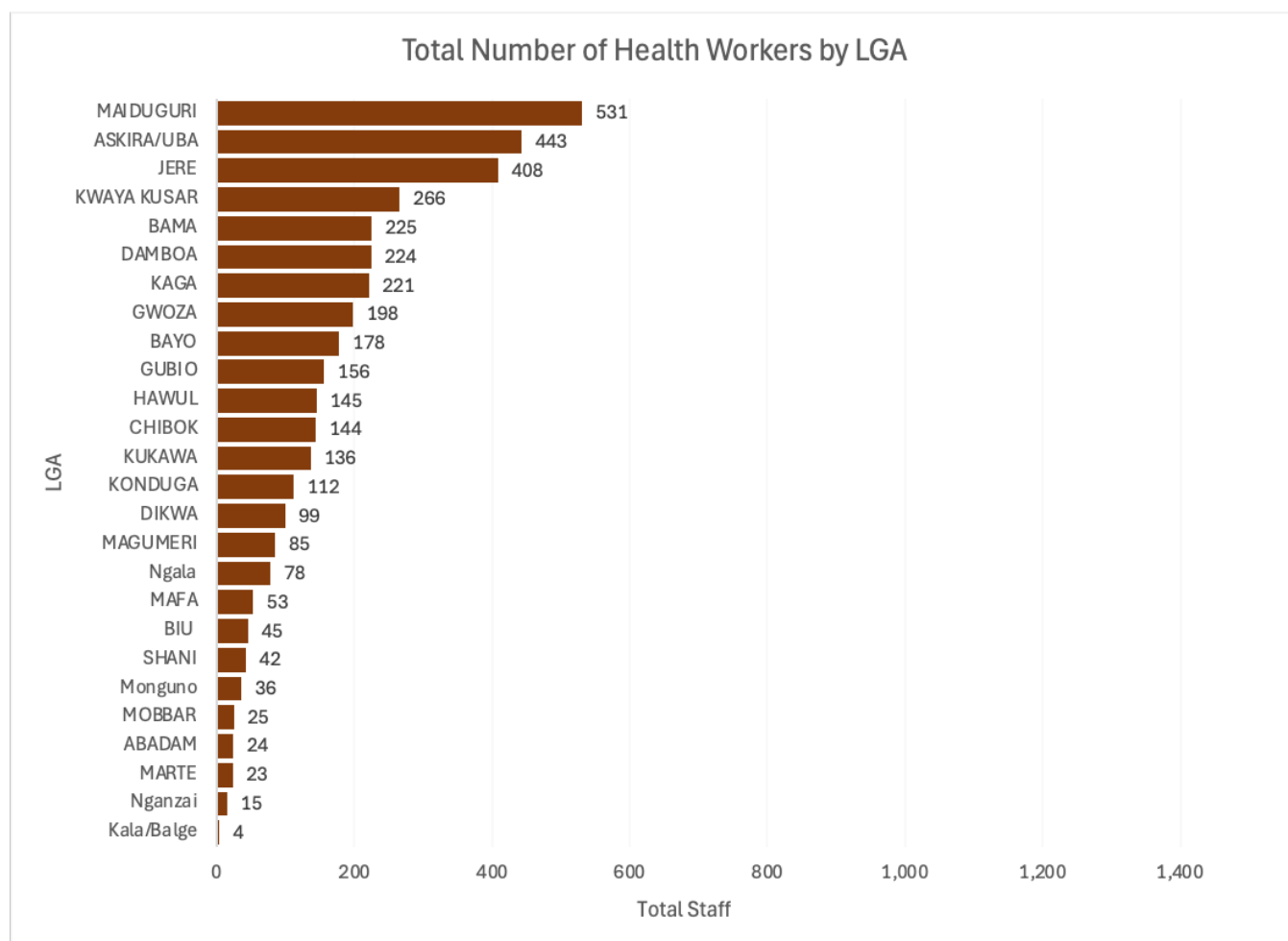


Figure 5: Total Numbers of Health Workers Per LGA in Borno State

Source: Borno State 2024 Human Resource for Health Profile

Staff-to-Facility Ratio Analysis

The staff-to-facility ratio provides a useful indicator of the average number of health personnel available per facility, serving as a proxy for workforce adequacy and distribution across Local Government Areas (LGAs). As part of its commitment to evidence-based workforce planning,

Borno State has analysed these ratios across all 27 LGAs to inform more equitable staffing decisions going forward.

The analysis reveals variation in staffing concentration across LGAs, which the State is actively working to address. Some LGAs — including Gubio (52.0), Kukawa (45.3), Biu (45.0), and Kaga (36.8) — record high staff-to-facility ratios, reflecting a strong staffing presence within their facilities. The State recognises that where this concentration is linked to limited facility availability, expanding geographic access to services will be a priority intervention. In LGAs such as Shani (1.2), Konduga (4.5), Monguno (2.1), and Hawul (6.0), lower staff-to-facility ratios have been identified. The State is committed to addressing these gaps through targeted recruitment and strategic redeployment to ensure that health workers in these areas are not overburdened and that communities receive the full range of PHC services they are entitled to.

With a State average of approximately 11.9 staff per facility, Borno State has a clear and actionable baseline from which to drive more equitable workforce distribution. The variation identified across LGAs does not represent a counsel of despair — it represents a roadmap. The State's recruitment, redeployment, and retention strategies, detailed in this report, are directly designed to close these gaps, strengthen facilities in underserved LGAs, and ensure that every community in Borno has access to an adequately staffed PHC facility.

2.1.5.2 Distribution of Health Workers by Gender

Borno State's PHC workforce reflects a commendable level of gender balance, with female personnel comprising the majority at 2,263 workers (58%) against 1,653 male workers (42%) across all 27 Local Government Areas. This is a positive indicator of the State's progress in building an inclusive health workforce and reflects deliberate efforts to encourage female participation in public health service delivery.

In urban LGAs, the data demonstrates particularly strong female representation. Maiduguri Metropolitan Council leads in this regard, with female staff (277) significantly outnumbering male

staff (254) — a reflection of the greater economic and professional opportunities available in urban centres and the State's success in attracting and retaining female health workers in these areas. A similar pattern is observed in Jere, where female workers (266) exceed their male counterparts (142), and in Askira/Uba, which also records a higher proportion of female staff.

The State acknowledges that in a number of rural LGAs, male health workers currently predominate. This reflects broader geographic, mobility, and sociocultural dynamics that influence deployment and retention patterns — factors that are not unique to Borno but which the State is committed to addressing as part of its equity agenda. Closing this gap is particularly important for maternal and child health service delivery, where the presence of female health workers has a direct and proven impact on care-seeking behaviour and health outcomes.

Building on the solid foundation of gender balance already achieved at the State level, Borno State will pursue gender-sensitive recruitment and deployment strategies that progressively extend strong female representation beyond urban centres into rural LGAs — ensuring that the benefits of an inclusive workforce are felt equitably across all communities.

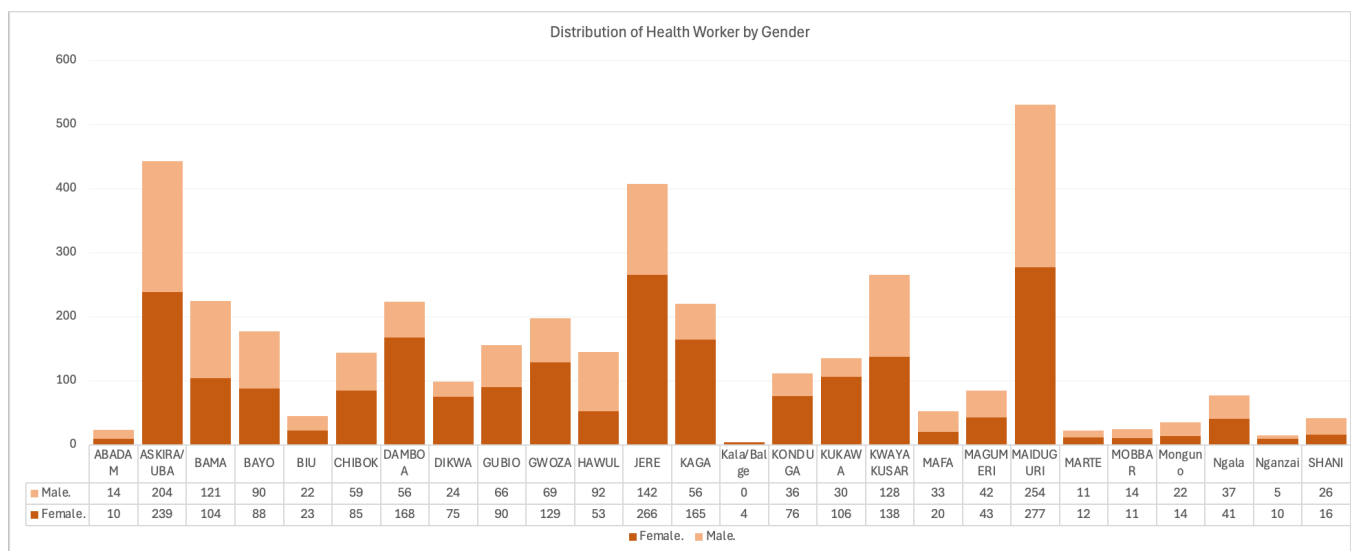


Figure 6: Distribution of Health Workers Per Gender
Source: Borno State 2024 Human Resource for Health Profile

2.1.5.3 Average Health Worker per Facility

The overall average staff per facility across the State is 12. The highest average is in Gubio LGA with 52 and this is followed by Kukawa LGA and the least in Shani with 1 as shown in Figure 7.

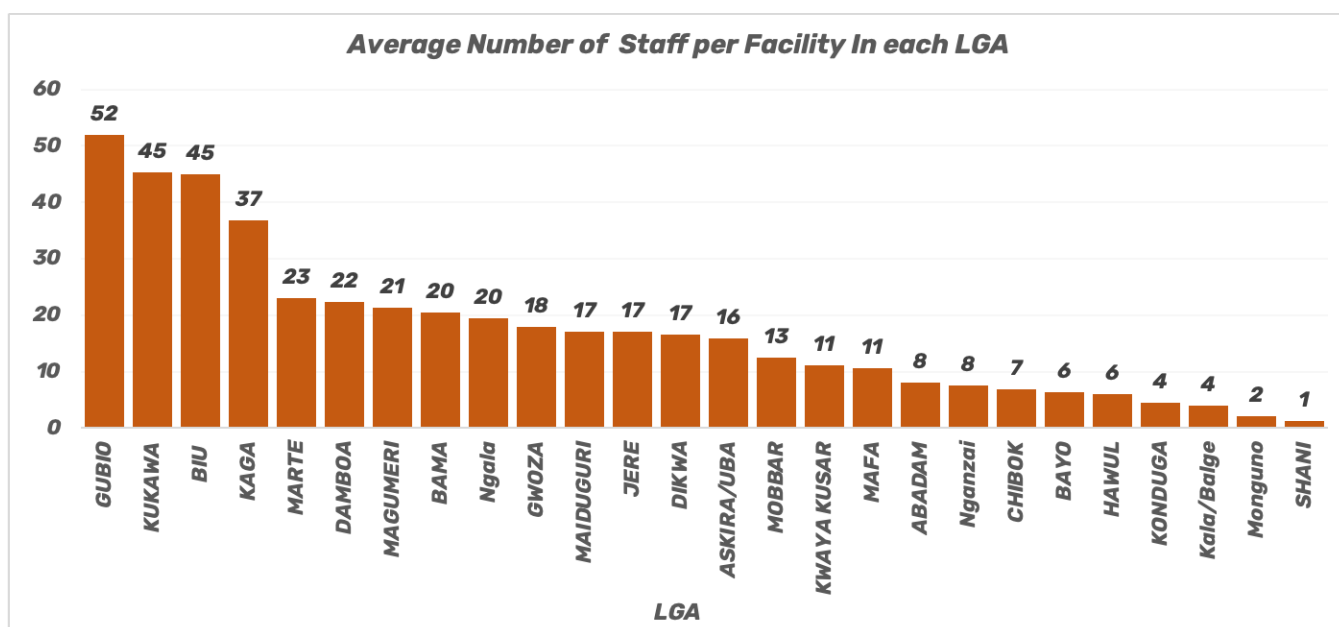


Figure 7: Average Number of Staff Per Facility

Source: Borno State 2024 Human Resource for Health Profile

2.1.5.4 Distribution of Health Workers by Job Area

Table 1 presents the distribution of health workers in Borno State by job area. Out of a total of 3,916 workers, 1,369 (35%) are classified as core health workers, including medical doctors, Community Health Officers (CHOs), Junior Community Health Extension Workers (JCHEWs), environmental health personnel, laboratory technicians, pharmacy technicians, and medical record officers. The remaining 2,547 workers (65%) are categorized as support staff, comprising administrative professionals, health assistants, health professional associates, health educators, and related cadres.

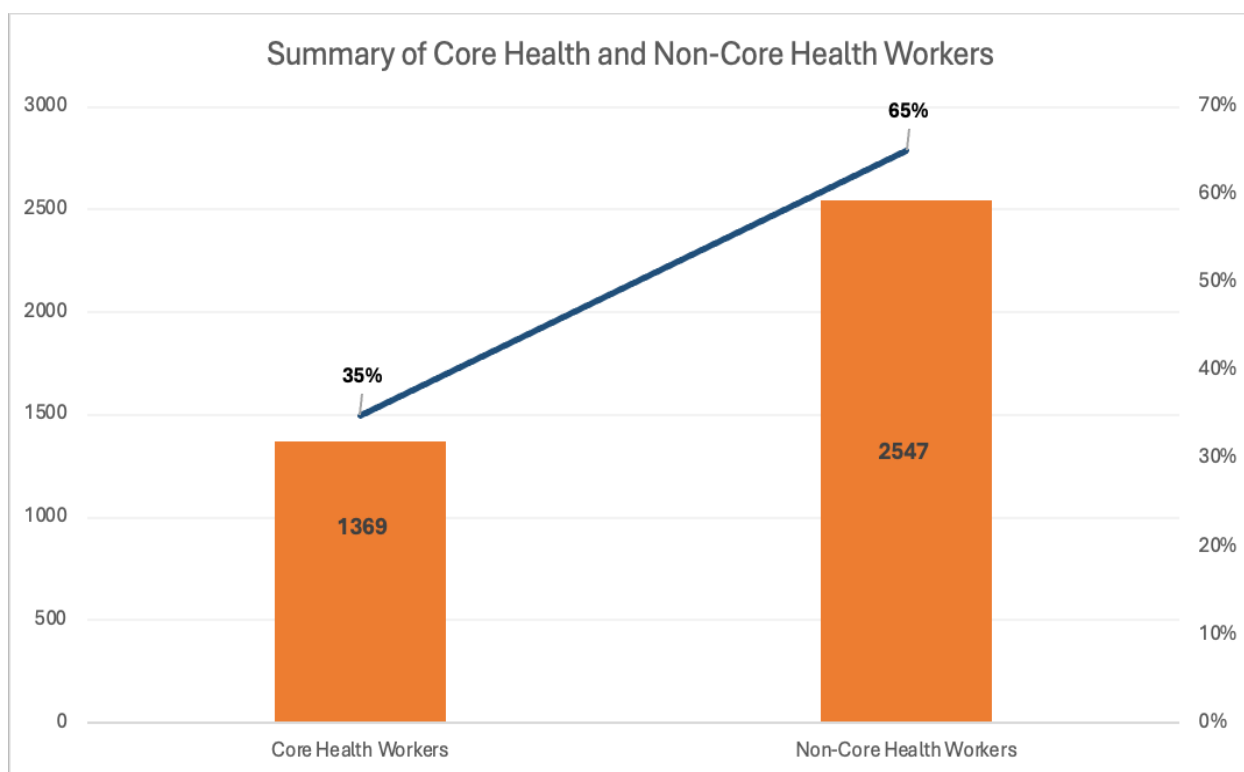


Figure 8: Summary of Core Health and Non-core Health Workers

Within the workforce, medical doctors, pharmacy professionals, CHOs, and nurses/midwives represent the smallest proportions, accounting for 0.001%, 0.001%, 0.015%, and 0.031%, respectively. The distribution of medical doctors across the Local Government Areas (LGAs) shows that only 4 out of 27 LGAs have at least one doctor. Bama LGA, Jere, Kukawa, and Kwaya Kusar LGA have one medical doctor each.

The distribution of nurses/midwives reveals that Abadam, Hawul, Kala/Balge, Mafa, MARTE , MOBBAR, Monguno, Ngala, Nganzai and Shani LGAs have none. All other LGAs have at least one nurse/midwife, with Jere recording the highest number at 26, followed by Askira/Uba LGA with 17. For CHOs, CHEWs, and JCHEWs, the total workforce stands at 1,198.

Table 1: Distribution of Health workers in Borno State by Job Area.

LGA	Community Health Officer	Community Health Extension Worker	Junior Community Health Extension Worker	Health Information Management	Medical Doctor	Medical Laboratory Scientist	Nurse / Midwife	Pharmacist	Total Core Health Workers	Total Support Health Workers	Grand Total
ABADAM	2	4	5	0	0	0	0	0	11	13	24
ASKIRA/UBA	2	109	62	3	0	3	17	0	196	247	443
BAMA	0	8	57	1	1	0	9	0	76	149	225
BAYO	7	42	36	5	0	0	1	0	91	87	178
BIU	0	5	4	0	0	0	2	0	11	34	45
CHIBOK	1	13	2	1	0	0	3	0	20	124	144
DAMBOA	2	23	8	0	0	0	9	1	43	181	224
DIKWA	3	22	12	2	0	0	2	0	41	58	99
GUBIO	4	41	20	2	0	0	12	0	79	77	156
GWOZA	2	25	46	3	0	0	2	0	78	120	198
HAWUL	1	28	18	0	0	0	0	0	47	97	144
JERE	12	70	47	12	1	0	26	0	168	240	408
KAGA	1	20	8	0	0	0	8	0	37	184	221
Kala/Balge	0	3	0	0	0	0	0	0	3	1	4
KONDUGA	2	24	12	3	0	0	2	0	43	69	112
KUKAWA	1	15	12	0	1	0	6	0	35	101	136
KWAYA KUSAR	6	56	39	0	1	0	10	0	112	154	266
MAFA	3	7	10	1	0	0	0	0	21	32	53
MAGUMERI	0	9	23	0	0	0	2	0	34	51	85
MAIDUGURI	6	84	63	8	0	0	11	0	172	359	531
MARTE	2	4	5	0	0	0	0	0	11	12	23
MOBBAR	0	1	0	0	0	0	0	0	1	24	25
Monguno	1	3	6	0	0	0	0	0	10	26	36
Ngala	0	1	0	0	0	0	0	0	1	77	78

Nganzai	0	6	2	0	0	0	0	0	8	7	15
SHANI	0	11	9	0	0	0	0	0	20	22	42
Grand Total	58	634	506	41	4	3	122	1	1367	2546	3,916

Source: Borno State 2024 Human Resource for Health Profile

2.1.5.5 Distribution of Health Workers by type of Duty Station (Rural-Urban Continuum)

As shown in Figure 9, most health workers in Borno State are deployed to facilities in the rural areas. Approximately 64% of the workforce serves in rural communities, while the remaining 36% are based in urban centers. Maiduguri Metropolitan Council (MMC) records the highest number of urban-based health workers, with 531 personnel, largely due to the higher concentration of facilities within the metropolitan area. Similarly, Jere has a significant urban workforce, with 408 health workers stationed in urban locations. In contrast, LGAs such as Kwaya Kusar, Askira/Uba, and Damboa have most of their health workers deployed to rural duty stations.

Overall, the distribution of health workers closely mirrors the spatial distribution of health facilities across the state. LGAs with a higher number of urban facilities tend to have more urban-based staff, while those with predominantly rural facilities have a corresponding concentration of personnel in rural areas.

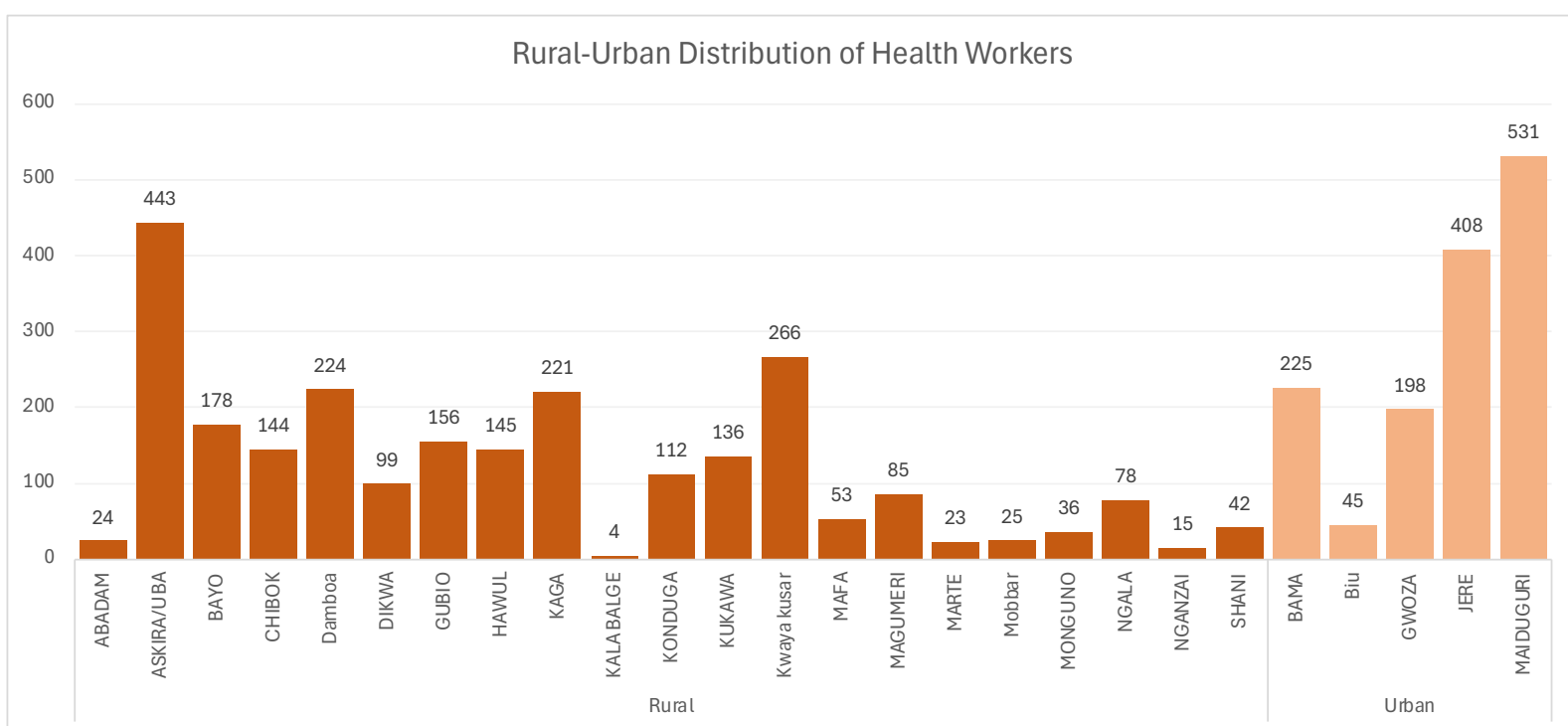


Figure 9: Rural-Urban Distribution of Health Workers

Source: Borno State 2024 Human Resource for Health Profile

2.1.5.6 Analysis of Workers Leaving the Service Between 2025 and 2029

Based on the retirement age of 60 years operational in the health sector of Borno State, a total of 758 out of the full-time staff are projected to leave the service between 2026 and 2030 through retirement. Specifically, 111 workers are expected to retire in 2026, another 111 in 2027, 175 in

2028, 168 in 2029, and 193 in 2030. This represents a significant outflow of experienced personnel over the next five years.

Table 3: Estimated Retirement from the Healthcare Workforce per Year

Year of Retirement	No of HWF Retirees
2026	111
2027	111
2028	175
2029	168
2030	193
Total Retiring in the next 5 years	758

Source: Borno State 2024 Human Resource for Health Profile

2.2 Operational Gap Analysis and Findings

2.2.1 PHC Staff Gap by Professional Area in Borno State

The gap analysis is informed by the current conditions on the ground in Borno State and reflects the State Government's commitment to realistic, achievable, and evidence-based planning. Of the State's 312 wards, 274 currently have operational PHC facilities — and it is on these 274 wards that the upgrading programme is anchored. The strategic objective is clear: to ensure that every one of these wards is served by at least one facility that meets the NPHCDA minimum standard of a functional, adequately equipped Primary Health Clinic. The remaining 38 wards, where operational presence has been disrupted by the security situation, are identified as a longer-term restoration priority.

In wards where more than one PHC facility currently exists, the State has adopted a focused and pragmatic approach — identifying a single facility in each such ward for priority upgrading to the required standard, while ensuring that all other facilities continue to operate and provide supplementary services to their communities. This approach maximises service continuity, keeps the upgrading programme focused and deliverable, and ensures that no community loses access to care during the transition.

Borno State understands the scale of the workforce investment required. Against the NPHCDA minimum staffing standard, the State's 329 PHC facilities across 274 operational wards face an

aggregate staffing gap of 2,860 workers. The State is equally prepared for the workforce transitions ahead — with 758 health workers projected to leave the service between 2026 and 2030 through retirement and natural attrition, succession planning is already embedded in the recruitment strategy.

Underpinning all of this is the State's recognition of its demographic trajectory. With a population projected to grow from 6.9 million in 2025 to 9.2 million by 2030 — driven by an annual growth rate of 3.98% and high birth registrations — Borno State is planning not just for today's gaps but for tomorrow's demands. This population growth will require not only the filling of existing workforce gaps but the progressive expansion of PHC infrastructure and staffing capacity to ensure that health coverage keeps pace with a growing and increasingly dispersed population. The State is committed to meeting that challenge with the same resolve and evidence-based approach that has guided this mapping exercise.

The staffing gap by professional area is shown in Table 4 below.

Table 4: Total PHC Staff Gap by Professional Area

PHC Personnel	Minimum No. per PHC **	Total No. Required across the State	Total No. available across the State	The gap across the State
Nurses/Midwives	4	1,096	122	974
CHO	1	274	58	216
CHEWs	3	822	634	188
JCHEWs	6	1,644	506	1,138
Lab Technicians	1	274	181	93
Pharmacy Technicians	1	274	23	251
Environmental Health	1	274	504	0
Medical Record Officer	1	274	955	0
Note: The total gap identified is based on the number of operational health facilities across 274 wards.				

***NPHCDA Minimum Standards for Primary Health Care staffing in Nigeria.*

Source: Borno State 2024 HRH Profile & Baseline Survey Analysis

2.3 Institutional Structure for Primary Health Human Resource Management in Borno State

The institutional structure for Primary Health Care (PHC) human resource management in Borno State is anchored by the Borno State Ministry of Health and Human Services, working in close collaboration with the State Primary Health Care Development Board (SPHCDB), Local Government Health Authorities (LGHAs), and newly established health sector boards. Together, these institutions oversee policy, recruitment, deployment, and supervision of PHC staff across the 27 LGAs and 329 PHC facilities

2.3.1 Minimum Standard Staffing Requirement for Primary Health Clinics (PHCs)

The minimum standard for PHCs in the State is presented in Table 6 below. The minimum standard was adopted from the National Primary Health Care Development Agency's minimum staffing standard for PHCs in Nigeria. Borno State is working towards achieving this minimum standard for both rural and urban health facilities in the next five (5) years. The same minimum staffing standards were retained for both rural and urban facilities.

Table 6: Minimum Standard for Primary Healthcare Staffing

Minimum Standard	Minimum No. per PHC
Medical Officer (If Available) **	1
Community Health Officer	1
Nurse/Midwife	4
CHEW	3
JCHEW	6
Pharmacy Technician	1
Environmental Officer	1
Medical records officer	1
Laboratory Technician	1
Support Staff (Health Assistant/Attendant (2), Security (2), General Maintenance Staff (1))	5

***NPHCDA Minimum Standards for Primary Health Care staffing in Nigeria.*

Source: National Primary Health Care Development Agency minimum staffing standard for PHCs in Nigeria.

2.3.2 Gender and Social Inclusion Requirements in Borno State

Borno State government is a gender-sensitive state, which is reflected in the state's inclusion of gender and social inclusion in its health policies and programmes. This is evident in the ratio of male to female healthcare workers, which currently favours the female gender. Hence, this

sensitivity will be considered in the employment and deployment of new health workers as set out in this report.

At the service-access level, Borno State has advanced its commitment to gender and social inclusion by implementing targeted community mobilization strategies and collaborating with Ward Development Committees, supported by partners such as WHO and UNICEF. These efforts focus on increasing women's participation in antenatal care, immunizations, and facility-based deliveries, while also engaging husbands and community leaders to foster greater acceptance of modern health services. By partnering with traditional and religious leaders, the state can address and reduce cultural barriers, ensuring that vital messages on maternal health and immunization reach the most vulnerable populations. Enhancements in healthcare infrastructure and the strategic deployment of female staff further improve accessibility for women and marginalized groups, collectively helping to overcome challenges related to cultural norms and physical access, and ultimately making primary healthcare more responsive and inclusive for all community members. These efforts will be sustained in the new recruitment and deployment drive.

2.4 Challenges of Health Workers Recruitment, Deployment and Retention in Borno State

Recruiting, deploying, and retaining health workers in Borno State face major hurdles, primarily driven by severe insecurity (Boko Haram insurgency), poor rural infrastructure (power, water, and housing), unattractive pay & benefits, weak governance/coordination, and sociocultural factors limiting female staff mobility, all leading to staff shortages and high turnover in some facilities, hindering effective health service delivery.

Insecurity stands out as a major factor leading to the above challenges. The fear of attacks, especially in rural and conflict-affected areas, discourages potential applicants from taking up posts in those localities. The impact of the Boko Haram insurgency did not only create a climate of fear but also led to widespread conflict-induced displacement, further complicating the deployment of staff to many regions. Remuneration remains a persistent issue, as uncompetitive salaries and allowances make rural healthcare jobs less appealing compared to urban or private sector opportunities. The lack of incentives, such as hardship allowances or bonuses, contributes to a general reluctance among health workers to accept challenging postings in remote locations. Deployment is further complicated by inadequate infrastructure as many rural areas lack staff housing, reliable electricity, clean water, and other basic amenities, making it difficult to attract and retain qualified personnel. Ineffective deployment policies often result in staff not being placed

where they are most needed, while security risks continue to threaten the safety of deployed workers, particularly in zones affected by conflict. Sociocultural barriers also play a role, with prevailing social norms, like those that requires wives not to work away from the locations their husbands reside, sometimes restricts the movement of female health workers to remote communities, despite efforts to promote gender and social inclusion.

Retention of health workers is undermined by unattractive working conditions, including dilapidated facilities, frequent drug shortages, and a lack of essential equipment, all of which contribute to staff frustration and low morale. Weak career progression, marked by limited opportunities for training, promotion, and professional development, further demotivates workers. Rural health workers often experience isolation due to weak supervision and support systems, also affecting retention. The combination of these factors has led to a "brain drain", with skilled workers leaving for better-paying jobs in cities or abroad.

In response to these challenges, the Borno State government is prioritising the development of targeted interventions aimed at improving the recruitment, deployment, and retention of PHC workers. Strategies include enhancing security measures in vulnerable areas, investing in essential rural infrastructure, offering more competitive and regular compensation packages, and providing clear pathways for career progression. Additionally, the government is strengthening governance structures to improve coordination and accountability across health facilities, while also implementing culturally sensitive policies that support the mobility and safety of female health workers. These efforts are designed to create a more supportive environment for health workers, reduce turnover, and ensure consistent, quality health services are available to all communities. Looking ahead, these initiatives will be integral to the state's broader workforce planning, ensuring that ongoing and future recruitment drives are both effective and sustainable as outlined in the subsequent recruitment and deployment plan.

3.0 RECRUITMENT/DEPLOYMENT PLAN AND COSTING

3.1 Recruitment Plan

The recruitment plan for Borno State, considering the projected annual population growth rate of 3.98% as reflected in the Borno State Health Profile (2025 edition). With the State's population recorded at 6,900,000 in 2025, the figures are expected to rise steadily over the next five years: 7,877,000 in 2026, 8,190,000 in 2027, 8,517,000 in 2028, 8,860,000 in 2029, and approximately 9,218,000 by 2030. This demographic expansion, combined with the anticipated retirements of health personnel between 2026 and 2030, will exert significant pressure on the primary health care workforce.

To address these challenges, the recruitment plan incorporates both the natural increase in population and the attrition of staff due to retirement. The annual population growth is projected to widen the staffing gap by an average of three additional workers in rural LGAs and four in urban LGAs each year, reflecting the need to meet new service demands. These incremental gaps have been integrated into the broader workforce gap analysis, ensuring that recruitment targets are realistic and responsive to the State's evolving health needs. By aligning recruitment with demographic trends and workforce attrition, Borno State aims to build a resilient primary health care system capable of delivering equitable and quality services across all 27 Local Government Areas.

3.1.1 PHC Workers Recruitment Plan by Job Area (5-year Projection)

The PHC workers' recruitment plan presented in Table 7 was developed based on the staffing gaps identified across job areas, the estimated number of staff due for retirement, and the projected impact of population growth on the primary healthcare workforce. The recruitment sequence follows a phased approach designed to balance human resource planning with cost efficiency, enabling the State to close its workforce gaps within five years. Accordingly, the State intends to recruit staff in annual proportions of 15%, 20%, 25%, 22%, and 18% between 2026 and 2030.

More clearly, Table 7 shows the five-year recruitment plan for Primary Health Care (PHC) workers in Borno State aims to close a total staff gap of 2,860 personnel across critical cadres. The plan prioritizes the largest shortages, beginning with Junior Community Health Extension Workers (JCHEWs, 1,138) and Nurses and Midwives (974). Recruitment will be phased and this plan reflects a deliberate strategy to strengthen the PHC system by frontloading recruitment in the most

critical cadres, while ensuring sustained additions through 2030 to stabilize service delivery across the State.

Table 7: PHC Workers Recruitment Plan by Core Job Area (5-year Projection)

PHC Workers Recruitment by Job Area	Total Staff Gap	Recruitment Plan				
		2026	2027	2028	2029	2030
Nurses/Midwives	974	146	195	244	214	175
CHO	216	32	43	54	48	39
CHEWs	188	28	38	47	41	34
JCHEWs	1,138	171	228	285	250	204
Lab Technicians	93	14	19	23	20	17
Pharmacy Technicians	251	38	50	63	55	45
Environmental Health Officer **	0	0	0	0	0	0
Medical Record Officer**	0	0	0	0	0	0
Total	2,860	429	573	716	628	514

**NPHCDA Minimum Standards for Primary Health Care staffing in Nigeria.

** No gaps (Surplus to be redeployed)

Source: Baseline Survey Analysis.

3.1.2 Workers Recruitment Plan by Classification of Duty Station

Most of the recruitment to be done will be channeled to fill the gaps at the rural duty stations as shown in Table 8 below. 77% (2,202) of the planned recruitment will be posted to rural duty stations while 23% (658) will be posted to urban duty stations to address the rural-urban disparities in the PHC workforce of the state.

Table 8. Workers Recruitment Plan by Classification of Duty Station

PHC Workers Recruitment by Classification of Duty Station	No. of Workers
Rural	2,202
Urban	658

Source: Baseline Survey Analysis.

3.1.3 PHC Workers Recruitment by Duty Station (PHC Facility)

PHC worker recruitment by facility will be done using the staff gap by facility.

3.1.4 Borno State PHC Staffing Recruitment Strategy

Borno State plans to employ the following strategies to fill the existing worker gap in the PHCs.

These strategies include:

- A. Targeted recruitment: Findings and gaps from the baseline mapping exercise will be used to generate a vacancy list to guide recruitment exercise with the most deprived locations prioritised, rather than blanket call for applications.
- B. Strategic Sourcing: Strategic investment to improve young people's admission enrollment various health training institution, thereby expanding the health manpower planning in the state. More importantly, coordinated efforts will be made to ensure quality of health service education in these institutions to provide the state with current and future right quality and competent health workforce for ready-made absorption into the PHC workforce of the state.
- C. Rolling Recruitment: because of the delays associated with mass government recruitment and deployment exercises, new recruitments will be made on a rolling basis (quarterly), to manage number and quality of so that new recruits can be absorbed without long delay.
- D. Retention-focused recruitment: To address the challenges of retention, particularly in rural duty stations, the government will collaborate with relevant health training institutions (like the Schools of Nursing, Midwifery and Health Technology, etc.) to sponsor candidates from underserved LGAs on the condition that they return to serve as PHC workers in their communities. In addition, modest hardship and rural posting incentives will be provided for staff that will be posted to remote or insecure areas (tied to their attendance).
- E. Leveraging National Schemes: As a stop-gap measure, the state will invest massively in housing, security infrastructures, facility renovation and equipping, and rural cum hazard allowances to maximise placements from the National Youth Service Corps (NYSC) for medical officers, nurses, laboratory technicians, and other critical health

- officers. The state will also partner with LGAs, community associations and faith-based groups to provide secure accommodation for the Corp members.
- F. Staff re-distribution: this will involve transferring staff from duty stations where they are surplus to duty stations with a shortage of such personnel. For example, in Table 7, the Surplus Environmental officer and Laboratory technician will be re-distributed to other health facilities within the LGA.
 - G. Transfer of Service: Opening the vacancy to existing qualified health workers in secondary healthcare facilities in the State for voluntary conversion.
 - H. Alternative Staffing Models: Use of contract workers when there are budget constraints, this can include re-engaging recently retired nurses and midwives on short contracts, particularly for training, mentoring and supervising junior staff. Furthermore, leveraging public-private partnership and NGO support to recruit and deploy qualified personnel until government systems can take full responsibility.
 - I. Recruitment of the outstanding gap through recruitment calls from externally qualified candidates after implementing the above strategies.
 - J. For continuous staff management, the State will intensify the use of technology and innovation to strengthen health workforce systems. This will cover workforce management, promotions, attendance monitoring, retirements, and redeployment processes, ensuring seamless coordination and improved responsiveness in the utilization of core PHC personnel. By leveraging digital tools and innovative practices, the State aims to enhance efficiency, transparency, and accountability in managing its primary health care workforce

3.2 Deployment plan

3.2.1 PHC Workers Deployment Plan by Classification of Duty Station

The planned recruitment efforts will primarily focus on addressing staffing gaps in rural duty stations, as highlighted in Table 8. This targeted strategy is intended to ensure that underserved communities receive the necessary PHC personnel to deliver effective and accessible healthcare services.

In addition, future workforce requirements have been projected using population estimates provided in Section 3.1. These projections indicate that a staffing gap of 2,202 personnel in rural

areas will be filled, while 658 personnel will be recruited to address shortages in urban centers by 2030.

These projected workforce needs have been fully integrated into the overall recruitment and deployment plan to ensure balanced coverage and sustained service delivery across both rural and urban areas. A detailed breakdown of the planned recruitment and deployment by duty station is presented in Table 9 below.

Table 9: PHC Workers Deployment Plan by Classification of Duty Station

Cadre	Duty Station	
	Rural	Urban
Nurse/Midwife	750	224
CHO	166	50
CHEWs	145	43
JCHEWs	876	262
Lab Technicians	72	21
Pharmacy Technicians	193	58
Environmental Health	0	0
Medical Record Officer	0	0
TOTAL	2202	658

Source: Baseline Analysis

3.2.2 Framework for Equitable Workers Deployment

To ensure equitable worker deployment, the State government will ensure priority is given to the rural areas with a minimum of 77% of annual recruitment deployed to the rural areas. To ensure the workers stay and serve at the duty stations they are deployed to (particularly the rural areas), all health facilities will be provided with decent work environment and working conditions/accommodation along with rural posting allowance/incentives. The government will also encourage indigenes to study health courses to serve their communities upon graduation.

3.3 Multi-Year Costed PHC Workers Recruitment and Deployment Plan

As depicted in Table 12 below, the state will require about ₦350.69 million in 2026, ₦545.13 million in 2027, ₦572.23 million in 2028, ₦210.57 million in 2029, and ₦221.04 million in 2030 to recruit the required primary healthcare workers from 2026 to 2030, train and deploy them, pay them minimum wage and allowances, and provide necessary office supplies and work tools/equipment to enable them function effectively. The estimated impact of inflationary trend and compliance with the National Minimum policy has been factored into these costs.

Table 10: Multi-Year Costed PHC Workers Recruitment and Deployment Plan

Cost Items	Description	Amount (N)				
		2026	2027	2028	2029	2030
New PHC workers to be recruited and deployed		429	573	716	628	514
Recruitment Exercise	Adverts, examinations, selection interviews, etc.	26,808,000.00	42,222,600.00	44,333,730.00	15,516,805.50	16,292,645.78
Onboarding Exercise	Orientation and pre-deployment training.	20,106,000.00	31,666,950.00	33,250,297.50	11,637,604.13	12,219,484.33
Personnel Cost	Salary, allowances, incentives for rural posting, etc.	134,040,000.00	211,113,000.00	221,668,650.00	77,584,027.50	81,463,228.88
Overhead Cost	Off-the-job Training, instructional materials & stationery, etc.	40,212,000.00	63,333,900.00	66,500,595.00	23,275,208.25	24,438,968.66
Capital Cost	Office space, furniture, etc.	107,232,000.00	168,890,400.00	177,334,920.00	62,067,222.00	65,170,583.10
Allowances & Incentives	Rural posting & redeployment allowance, etc.	15,589,120.08	17,351,880.12	18,055,590.12	16,609,365.54	17,385,205.82
Other Costs	Contingency, etc.	6,702,000.00	10,555,650.00	11,083,432.50	3,879,201.38	4,073,161.44
Total Cost		350,68G,120.08	545,134,380.12	572,227,215.12	210,56G,434.2G	221,043,278.00

Source: Baseline Survey Analysis and Financial Estimation.

3.4 Recruitment Calendar

Based on the multi-year-costed primary health care workers recruitment and deployment plan shown above, the state will implement the following recruitment calendar to ensure coordination and seamless implementation of the requirement and placements exercise across the state:

Table 11: PHC Workers Recruitment Calendar

ACTIVITIES	2026	2027	2028	2029	2030
Finalize and obtain approval for the recruitment and deployment plan					
Constitute a recruitment and deployment committee in collaboration with State and Local Government Civil Service Commission.					

Place adverts for recruitment in newspapers, on website, notice boards and social media					
Screen and streamline applications					
Conduct recruitment examination					
Shortlist successful candidates					
Conduct oral and verification of certificates					
Publish list of successful candidates with duty stations					
Conduct orientation, placements and pre-deployment training					
Conduct annual monitoring of attendance and service delivery quality, as well as performance appraisal exercise					

3.5 Integration with the State Health Annual Operational Plan

The findings and recommendations of this PHC baseline mapping exercise are designed to feed directly into Borno State's Health Annual Operational Plan (AOP). By providing credible data on the current workforce, staffing gaps, and projected needs, the baseline serves as an evidence base for setting annual priorities, resource allocation, and performance targets within the AOP framework.

Specifically, the recruitment and deployment projections outlined in this report will inform the human resource component of the AOP, ensuring that staffing plans are realistic, cost-costed, and aligned with available fiscal space. The baseline also highlights disparities in rural–urban distribution, gender balance, and cadre availability, which will guide equity-focused strategies in the AOP. Furthermore, the assumptions on population growth, retirements, and security improvements provide a forward-looking lens for planning, allowing the AOP to anticipate future demands and adjust accordingly.

By embedding the baseline findings into the AOP cycle, Borno State will further strengthen accountability, monitor progress against staffing targets, and ensure that investments in primary health care workforce development are systematically tracked.

4.0 RECOMMENDATIONS

Based on the findings of the baseline exercise, the following recommendations were made.

- Surplus PHC workers in facilities with surpluses should be redistributed to the facilities with shortages.
- It is essential to implement a strategic recruitment and deployment plan to ensure that healthcare workers are equitably distributed across all primary healthcare facilities, particularly in underserved rural areas.
- Improving the working conditions, providing adequate incentives, security, and fostering opportunities for professional development will help enhance the motivation and retention of healthcare personnel.
- Strengthening the monitoring and evaluation framework will enable the state to track progress, address emerging challenges, and refine strategies in real time.
- Budgetary allocation to the primary health sub-sector should be increased to accommodate necessary recruitment to close the human resource gap identified in the system.
- The principle of Primary Health Care Under One Roof (PHCUOR) should be aggressively pursued in the state.

By prioritizing these measures, the state can bridge critical gaps in the primary healthcare workforce and lay the groundwork for sustainable improvements in healthcare delivery

5.0 CONCLUSION

The baseline mapping exercise for primary healthcare workers in Borno State has highlighted substantial deficiencies in the human resources necessary to provide quality primary healthcare services throughout the state. These findings emphasize the urgent requirement for well-planned interventions to address shortages of healthcare personnel, with particular attention needed in rural areas where most of the population resides. The data compiled in this report offers a detailed overview of the status of the primary healthcare workforce in Borno State and establishes a foundation for future planning and decision-making.

To ensure effective delivery of primary healthcare services, it is imperative that the Borno State Government promptly addresses the identified workforce gaps. Key actions include increasing budgetary allocations to the health sector, centralizing management of primary healthcare activities under the Borno State Primary Healthcare Development Board and establishing a comprehensive recruitment and deployment plan. Furthermore, efforts must be directed toward enhancing the working conditions and incentives for healthcare workers, especially those serving rural communities, to support retention and motivation.

The recommendations provided in this report serve as a strategic guide for strengthening the primary healthcare system in Borno State. By systematically addressing human resource shortages and improving overall management of the healthcare workforce, it will be possible to ensure that every resident of Borno State has access to the quality healthcare services they need. Achieving these goals will require sustained collaboration and commitment from all stakeholders, including government authorities, healthcare professionals, and the communities they serve. Through collective effort, a healthier future for Borno State can be realized.

Annex

Annex 1: Baseline Survey Team

S/N	Team	Role
1.	Permanent Secretary, Ministry of Health and Human Services	Chairman
2.	Executive Secretary, Borno State Primary Healthcare Development Board (BSPHCDB)	Co-Chairman
3.	Director Planning, Research, and Statistics, Ministry of Health and Human Services	Secretary
4.	Director Planning, Research, and Statistics, BSPHCDB	Assistant Secretary
5.	Desk Officer of HOPE-PHC	Member
6.	Directors of Primary Health Care of 4 LGAs (One per Senatorial District & MNC)	Member
7.	HRH Focal person, BSPHCDB	Member
8.	HRH Focal person, Ministry of Health	Member
9.	Representative, Borno State Union of Health Workers	Member

Annex 2: Data Collection Tool

A. Name of your LGA:

B. The population of your LGA:

C. Baseline Survey Form

Name of Worker	Duty Station (Facility)	Gender	Cadre	Date of First Appointment	Type of Appointment (Permanent, Contract or Volunteer)	Date of Retirement	Qualification	Staff ID